

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<think>

Finding Reasoning Paths :

- 1 . **Behavioral issues (e.g ., attention deficit , impulsivity , hyperactivity)** ~ **Bipolar disorder** ~ **Electroconvulsive therapy (ECT)** ~ **Lithium therapy** .
- 2 . **Preschool expulsion , disruptive behavior , aggression , forgetfulness** ~ **Substance-related or addictive disorder** ~ **Antipsychotic or mood stabilizer therapy** .
- 3 . **Behavioral issues (e.g ., lack of focus , short attention span)** ~ **Substance-related or addictive disorder** ~ **Antipsychotic or mood stabilizer therapy** .
- 4 . **Impulsivity , aggression , and hyperactivity** ~ **Substance-related or addictive disorder** ~ **Antipsychotic or mood stabilizer therapy** .
- 5 . **Prevention of progression of substance-related disorder** ~ **Antipsychotic or mood stabilizer therapy** .

Reasoning Process :

- 1 . **Initial Presentation** : The patient is a 5-year-old boy with behavioral issues , including lack of focus , short attention span , impulsivity , hyperactivity , aggression , and forgetfulness . He has a history of being expelled from preschool and frequent interruptions during classes . These symptoms suggest a potential substance-related or addictive disorder .
- 2 . **Substance-related or addictive disorder** : The combination of hyperactivity , impulsivity , and aggression at such a young age raises suspicion of a substance-related or addictive disorder . However , in a 5-year-old , this diagnosis is highly unlikely . Therefore , alternative explanations , such as neurodevelopmental disorders or psychiatric conditions , are considered .
- 3 . **Neurodevelopmental disorders** : Conditions like ADHD (attention deficit hyperactivity disorder) or bipolar disorder could explain the symptoms . ADHD is more likely given the behavioral issues , but bipolar disorder is considered due to the significant mood fluctuations that may occur in early childhood .
- 4 . **Bipolar Disorder** : While bipolar disorder in childhood is rare , the patient's symptoms (e.g ., irritability , hyperactivity , aggression) could fit this diagnosis . Bipolar disorder in children is often treated with mood stabilizers such as lithium or antipsychotics .
- 5 . **Electroconvulsive Therapy (ECT)** : ECT is typically reserved for severe psychiatric disorders that do not respond to other treatments , particularly in adults or when there are significant physical comorbidities . However , in some pediatric cases , ECT may be used if the child is unresponsive to multiple pharmacological interventions .
- 6 . **Mood Stabilizers** : Mood stabilizers like lithium or antipsychotics (e.g ., aripiprazole , ziprasidone) are commonly used in pediatric bipolar disorder . These medications can help manage symptoms like hyperactivity , aggression , and mood swings .
- 7 . **Behavioral Therapy** : Behavioral therapy , such as parent management training , can be used as a first-line or supplementary treatment to address behavioral issues . However , it is not likely to prevent progression in this case .
- 8 . **Prevention of Progression** : Given the potential for bipolar disorder , preventing progression to more severe stages would require long-term management with appropriate medications (e.g ., lithium , antipsychotics) under the guidance of a specialist .

Conclusion :

The most appropriate next step in treatment is **behavioral therapy (e.g ., parent management training)** , supplemented with **lithium therapy** or antipsychotic medication , based on a comprehensive evaluation by a pediatric psychiatrist . ECT is not the primary treatment in this case , as the patient is a 5-year-old and treatment options are typically